

Natalie Aguilar
Exercise 2.10
April 23, 2025

YouTube Video Link

<https://youtu.be/ZikeLj7EFkk>

Tableau Link

https://public.tableau.com/shared/NMMFCJX9F?:display_count=n&:origin=viz_share_link

Were there any limitations that prevented you from conducting an analysis?

Yes, there were a few limitations that prevented a deeper analysis. The main issue was the lack of detailed, individual-level data. The dataset only provided total deaths by age group, without including other potentially significant factors such as pre-existing health conditions or vaccination status. These details would have allowed for a more targeted analysis. For example, if I had access to data, I could have examined how pre-existing conditions interact with age to influence mortality.

Did your data have any limitations that may have affected your results?

Yes, the dataset likely had limitations that could affect the results. One concern is the potential for data bias, particularly in how deaths are reported or categorized. Influenza-related deaths in younger age groups may be underreported or misclassified, leading to an underestimation of mortality in those populations. There is also a lack of context around the data source itself, such as how the data was collected and whether it is complete or representative of the entire population. Without clarity on these aspects, there's a risk that the analysis reflects incomplete or skewed information, which could impact the accuracy and reliability of the insights.

How might you monitor the impact of the staffing changes you recommended?

If staffing changes were implemented to better support older populations, such as increasing the number of care specialists or boosting flu prevention efforts, monitoring their impact would be essential. This could be done by tracking key outcome indicators over time. For example, comparing mortality rates in the 65+

age group before and after the staffing changes could help evaluate effectiveness. Additionally, other indicators like hospitalization rates, ICU admissions for flu-related complications, and the speed of patient care delivery could offer a broader view of how the staffing changes are influencing health outcomes.

Is there a metric that could be used for monitoring this impact?

Several useful metrics could help monitor the impact of staffing changes. One primary metric would be the influenza-related mortality rate among people aged 65 and older. A decrease in this rate over time could suggest that the staffing adjustments are having a positive effect. Other helpful metrics include vaccination uptake rates within the older population, the average time between diagnosis and treatment, and patient satisfaction scores specifically among elderly patients. Monitoring these indicators consistently would allow for data-driven evaluations and adjustments to healthcare strategies as needed.